Apalutamide: prostate cancer with ongoing ADT

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Continuous oral treatment

Active medicines: Apalutamide

Emetogenic risk: Minimal to low (oral)

Medicine	Dose / route	When
Antiemetic plan for current oral dose	Individual prescription	As agreed with oncology/pharmacy Review QT prolongation and all interactions; no automatic intravenous (IV) chemotherapy script.

Additional phase-specific support

- Take at the same time each day, with or without food. Swallow tablets whole with water. If a dose is missed, take it as soon as possible that day and return to the usual schedule the next day; never take extra tablets to make up a dose. If you cannot swallow the tablet, a pharmacist must teach the correct preparation for your 60 mg or 240 mg formulation or feeding tube.
- Repeat blood count, kidney and liver tests on the schedule agreed with your team. Thyroid function and electrocardiogram (ECG) as clinically indicated. If warfarin is unavoidable, international normalised ratio (blood-clotting test) (INR) weekly until the dose is stable. The NCCP regimen does not set one fixed interval for every routine blood test.
- Report rash promptly. Blisters, peeling skin, fever with rash, or mouth/eye sores need immediate medical advice. New or worsening cough or breathlessness requires urgent assessment for lung inflammation. Seizures require emergency help. Discuss driving and medicine interactions.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____